

Death's Broker: the Ethics Consultant in the ICU

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Modern Western medicine may be seen as an effort to wrest the phenomenon of healing away from religious communities. Peter Berger and Jonathan Imber have proposed that it might be especially fruitful to investigate the encounter of the religious pluralism that has persisted thorough the modern period with the full-blown secularization as both are found in the contemporary Western hospital.¹

Speaking as someone who does what is called “ethics consultation” in such an institution, I offer four clusters of ideas to further this investigation. I will present them as follows:

- 1) The ethicist's role as consultant in hospital is best understand as brokering the progress of technical death and bodily death in challenging cases, conducting a variety of interlocking negotiations, so that the two deaths occur at the same time, to the degree that that is possible;
- 2) From that standpoint, built out of dozens (or hundreds) of such negotiations, the ethicist who is *looking*², will be in a unique position to see and say useful things about secularization and religion in the hospital. This is in significant part because a working *knowledge of both religious and secular pluralism is essential for one to succeed as a broker of a “good death” in a tertiary care medical facility*. And what might these “useful things” be?

3) Secular pluralism

- a. There is a pluralism of secular discourses in our hospitals, such that, for instance, people speaking “respiratory therapy” cannot be guaranteed to understand people speaking “accounting”—and vice versa.
- b. Even so, we can identify “technical body talk” as the most privileged secular language of hospital.

4) Religious pluralism

- a. Religious practice and belief enter the hospital with and through the physical body;
- b. There exists a phenomenon I will call “negotiated transcendence”, which is often the only shareable route to the sacred within the confines of the hospital. And that the ethicist may be called on to negotiate this as well.³

As death's broker, the ethics consultant must perforce be multi-lingual, in a rough-and-ready way, because so much of negotiation takes place at the sticking point of secular languages (and their various dialects) with one another, religious languages (and their various dialects) with one another, and the tongues of secularization and religion with one another—all of which one finds, all the time, in tertiary level hospitals.⁴

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³ Wendy Cadge's comments on negotiation and spirituality helped me refine the formulation of “negotiated transcendence”. (Cadge 2012: 14–17 *et passim*.)

⁴ I deliberately say negotiator or broker, and not translator, because the latter would obviously be impossible—too many languages, way too many dialects. True enough, negotiation does involve translation in some sense—but I would argue, mostly by way of pidgin variants of the major secular and religious languages commonly found in hospitals.

I

Recent ethnographical work has provided rich insights into the working of intensive care units (ICUs). Key texts include the works of Renée Anspach (1987, 1993), Bosk (2003),

Cadge (2012); Chapple (2010); Kaufman (2005), Mesman (2008); Seymour (1999, 2000, 2001), and Zussman (1992). While I am going to draw heavily on two papers by Seymour to frame my discussion, all these studies lie in the background of what follows.

In her papers “Negotiating natural death in intensive care” (2000) and “Revisiting medicalisation and ‘natural’ death” (1999), Seymour makes a distinction between bodily dying and technical dying.

. . . complex strategies [are] employed by medical staff during the withdrawal of medical treatment in intensive care to ensure that alignment is achieved between the potentially divergent trajectories of the “seen” bodily dying, which is recognized on the basis of clinical experience; and the discursive negotiation of “known” technical dying, which is informed by data from blood results, monitoring equipment, and investigations.

And she argues that a “good death” in the ICU occurs when the death of the one body “coincides” with the death of the other.

Containing and preventing divergence between “bodily” and “technical” dying are represented as “nature taking its course” and “natural death” by medical staff in intensive care. (Seymour 1999: 694)

Seymour works with this basic dichotomy to characterize negotiations medical teams make essentially with themselves to understand and achieve “natural death”. But to capture the involvement of those parties interested primarily in the patient as a person into these negotiations towards “natural death” in a full way, we have to take this analysis several steps further.

Despite appearances, neither bodily death nor technical death is a discreet “event”. Despite appearances, neither the “seen” body of the patient nor the “known” body of the technicians is a singular “object”. Both pairs are themselves complex and negotiated entities.

We might, for example, speak of the technical body as a cluster of interests, and technical dying as something accomplished by multiple agents. In the ICU, one might list the primary team, critical care team, and respiratory therapists as primary stakeholders in the “known” or “technical body”—as well as agents in the technical dying.⁵ And recognize in turn

⁵ In this vein, we might say that the technical body is presented in particularly pure form in the daily rounds that take place in ICU’s.

that each of these three stakeholders will likely have interests distinct from those of the other two.

Likewise, we could speak of the patient’s body as a cluster of interests, and bodily dying as something accomplished by multiple agents. For the patient’s body, the “seen” body, family and friends, the readers of the signs of bodily dying (whether lay or professional), along with religious communities, could be seen as primary stakeholders—and in turn all of these understood as acting in concert to accomplish a bodily dying. And we would recognize that each of these stakeholders is likely to have interests that diverge from those of all the others.

Extrapolating from this further analysis, we come to realize that, for a *fully satisfactory* “natural death” to occur, *all the interests of all members of these groups must be aligned*. The family must be “ready”, the primary team and the critical care team must be in agreement, and all religious expectations addressed.⁶

What then constitutes the “natural death” that is so desired—not just by medical teams, but by families and companions of the dying as well? How can we tell that a “natural death” has been achieved? Seymour once again:

. . . firstly, that the process of dying was neither too prolonged nor too precipitate.

The patient can linger on and on, well past the time the critical care team (the team running the ICU) thinks appropriate—in spite of the fact that the primary team (for example, “surgery”, or “medicine”, or “neurology”) responsible for the patient’s hospital course “isn’t ready.” The patient can die before anyone is ready—family or team.

Secondly, that “technical death” could be aligned to the “bodily death”, thereby legitimating the medical control of events and ensuring that death only occurred once all avenues of possible treatment were exhausted.

The body may be on technical grounds “very near death”, when the patient’s body itself doesn’t yet show many or any of the common clinical signs of “active dying”: changing

⁶ My colleague Joe Fanning has pointed out that families have their own interpretations of the “technical body”, though talk about it is likely not to be (in his felicitous phrase) “their preferred nor predominant idiom”. One might adduce as evidence this scene in a children’s hospital: The dad is sitting in that severe kind of recliner found in hospitals watching the numbers and the graphs on the monitor as if he were at home in front of the TV. Meanwhile the mom is in the rocking chair studying her tablet, researching terms and numbers that have been thrown at her by the teams during the day.

In terms of the argument I am developing in the text: Does this make the family a potential “interested party” in technical dying—just as a clinician who knows a patient personally might be an “interested party” in bodily dying? Yes. But lesser shareholders, if you will—on each side.

breathing patterns, temperature changes, altered skin color and tonality.

Thirdly, that [the patient's] awareness of dying was suppressed and her family's acceptance of death facilitated.

Medication is given to prevent anxiety, relieve pain and "reduce suffering". At times patient may appear to the team to be suffering but seems to be improving to the family—or vice versa. The family is often, in the teams' language, "not ready".

Lastly, technological control engendered an environment in which the family was able to witness a gradual, quiet and dignified event. (Seymour 1999; 697–698)

Yet sometimes death is not gradual, not quiet, not dignified. And this can be hard on everyone.

Given the complexity of these criteria, it is not hard to see that things will often go awry. How then is the convergence of bodily dying and technical dying engineered by medical teams and their consulting services? Seymour:

However, the data suggest that "natural death" is primarily constructed during medical work by means of four interactional strategies.

... firstly, the establishment of a "technical" definition of dying—based on blood results, monitoring evidence and investigation—over and above "bodily" dying, based on the habitual recognition of the senses informed by clinical experience. Such definition occurs across a variable time period and involves the active negotiation and re-negotiation of the meaning of medical-technical data.

A given medical team focused on one organ system will interpret the significance of a sequence of lab values differently from another. The liver numbers may be improving as the kidneys fail. The severity of brain damage shown in an MRI can be read very differently by the neurology team, the trauma team and the critical care team. Nurses at the bedside work diligently, hour-by-hour, to correlate the numbers they recite at rounds and the target values established then, with what their clinical experience tells them about the body they take care of all day long.

Secondly, the alignment of the trajectory of "known" technical dying with the development of "seen" and "felt" bodily dying. It is this alignment which ensures that the events of treatment withdrawal are seen to have no directly causative link to death.

"There is nothing else we can do." "She is showing us what we need to do next." The technical activities associated with

dying—procedures, vent settings or extubation, medications withdrawn and medications introduced, whether or not to introduce antibiotics or begin dialysis—are not just to be coordinated with the bodily "activities" of dying—changes in breathing and heart function, in skin color and temperature. Their convergence will also preferably occur in such a sequence and such a manner that the technical can be experienced and shown to be following the lead of the bodily as death approaches.

Thirdly, the problem of causation is further dealt with by a strategic balancing of medical action with medical non-action. This allows for a diffusion of responsibility for death to the body of the ill person.

A transition from the dominance of the language and activities that constitute technical dying to those that constitute bodily dying takes place as aggressive care (i.e., those interventions guided largely by specialized, technical information) take a backseat to those of comfort care (i.e., those interventions keying off the bodily dying process). One kind of action is gradually substituted for another.

Lastly, medical work is directed, where circumstances allow, towards the incorporation of patients' companions and nursing staff into the decision making process. (Seymour 2000; 1245)

"You are the ones who finally have to make the decision." "Only you know what your father would want us to do now." The medical teams which have drafted and directed the hospital course, as death nears, step back to share responsibility with the families and companions of the dying.

All these strategies serve to bring together all the "interests" that enact the technical body and the patient's body together in such a way that all can agree that the death is occurring *ought* to be taking place—in two senses of "ought", the moral as well as the consequential. This "ought" in turn allows everyone to take up confidently their responsibility for their part in orchestrating the event.

Given the very high desirability of a "natural death", a "good death" for medical teams and for families, it will be prudent, if not essential, to identify before crises occur who it is that can best help engineer "natural death" in cases where technical dying and bodily dying appear to be diverging rather than converging. Who to call when the best efforts of the medical teams involved to pull them back together are failing? In larger hospitals and tertiary medical centers there are increasingly two primary options: the palliative care team and the ethics consultation service. Let us call the latter "Ethics" for short.⁷

⁷ It is not uncommon to hear as one walks onto a unit: "Ethics is here." How bizarre. And unsettling. Further: It is customary in our service to sign our notes in the EMR with "Thank you for calling Ethics."

Why, we should ask, these two services and not other long-standing ones? We give Palliative Care and Chaplaincy their own treatments below. But why not Social Workers, Nurse Practitioners, Nurse Managers, Charge Nurses, Bedside Nurses? The people in these roles are constantly moving back and forth between medical teams and families, interpreting the relationship of the technical body to the patient's body. Why are these clinical roles not the ones designated to conduct these types of ICU negotiations? Why is this sort of negotiation not in their "scope of practice"? Here we want the light sociology and history can shed on these roles. Each has typically involved education that was considered less "formal" and "scientific"; had assignments that were considered "less prestigious"; had lower pay; and were held by women.⁸ These roles also had fairly established institutional identities and functions by the time the need for the negotiations we are talking about became so necessary.

Let's take a stab at answering the question of why "Ethics" is a consulting service at all, and along with it Palliative Care. Put as succinctly as possible: "The Vent invented Ethics." That is: My proposal for the etiology of ethics consultation is that the medical ventilator's capacity to sustain life made the ICU possible. And that the ICU itself—and its attendant phenomenon, "negotiated death"—*created* the ethics consultant. The ICU called forth the role out of law, medicine and chaplaincy.⁹ *Not* primarily because big decisions have to be made there, though some certainly are. But rather because a very specific type of negotiation has to take place in ICUs and no existing services were in a position to provide it.¹⁰

This novel situation has called forth the roles of *both* Ethics and Palliative Care. In theory, people filling these roles can approach physicians roughly as peers, and have a back- and-forth function that is still being shaped. They both operate outside clear expectations.¹¹

⁸ I would hasten to add that I consider the people filling these roles in the hospitals where I have worked to be the heroes of the healthcare these institutions offer.

⁹ For a brief look at the history of the ICU see Reiser (1985). The idea that a technological space might "create" a role, might "enact" a feature of the social, is drawn from Actor-Network Theory. See especially: Latour (2002, 2011); Law (1992); and Mol (2003).

¹⁰ This perspective on negotiation has at least as much explanatory power for the rise of ethics consultation as do the "origin myths" propagated by bioethicists. See Gains and Juengst (2008). I would see my account as supplementing sociological and historical perspectives on the field. Required reading here for bioethicists being of course the works of Renée Fox and Judith P. Swazey. Essential would be "Medical morality is not bioethics—medical ethics in China and the United States". 1984. (And then also Fox and Swazey 2005, 2008).

¹¹ People with many kinds of training and experience can fill the role of ethicist: M.D.'s, nurse practitioners, nurses, social workers, lawyers, chaplains and philosophers. There is every reason to be skeptical that this last group will have any special advantages over the others when it comes to filling the Ethics role successfully. Indeed, there is much reason to suspect the contrary.

This relatively free rein is at once advantageous and not. For, as the briefest amount of time in the ICU demonstrates, a significant portion of attending physicians does not consider palliative care teams or ethics consultants to be peers. We are convenient in bad spots, perhaps in large part because we are fresh to the scene. Our authority remains a function more of charisma than institutional anchoring—as yet.¹² Where I am employed, Ethics and Palliative Care work hand in glove. These negotiations as the end of life approaches are so often so convoluted, so messy, the stakeholders so diverse, that frankly one needs all the help one can get. ("Why did you call Ethics for this case?" I say to my colleagues in Palliative Care. Often enough the answer is: "To share the love.") Some interested parties respond better to the white coats of Palliative Care, others to the street-dress of Ethics. Some to the bias towards technical death of Palliative Care; some towards the bias towards physical death of Ethics. Sometimes we play good-cop, bad-cop. It will be interesting to watch the evolution of this. As far as this negotiation function goes, it may well be that Palliative Care displaces Ethics in the next decade or so.

What does the back-and-forth of a representative ethics consultation look like? Consider the fact that "Ethics" is commonly consulted to assist teams in the implementation of the "four interactional strategies" Seymour delineates and that we discussed just above. To wit:

- 1) Differing interpretations and constructions of "known" data and "seen" signs and their relationship to one another result in disputes within or between or among medical teams that in turn can spur a call to "Ethics".
- 2) "Ethics" may be called in to lead or participate in a discussion of what is called "goals of care". This phrase may be translated as: How what is being done by clinicians to the "technical" body fits with what anyone can see is happening to the patient's body.
- 3) "Ethics" is often called in to facilitate discussions about balancing medical action and medical non-action, about withholding or withdrawing care.
- 4) "Ethics" may be called to weigh in on who should be involved in shared decision-making, by helping assess who appropriate surrogates are and what their role ought to be. Especially as regards "the preferences of the patient" and her or his perceptions of "quality of life". Both of which the surrogates presumably (hopefully!) share with the patient. "Ethics" is also often called on to facilitate the discussions necessarily involved in such

¹² Not that there isn't formal institutional structure for ethics consultation—there is. But the range of approaches, the shifting expectations, and the wide variety of training and backgrounds mean that much of the purchase one has walking into a unit is local and situational, rather than bureaucratic. The classic treatment of charisma and institutionalization is of course Max Weber's.

shared decision-making processes, particularly when such discussions are stalled or acrimonious.

I adduce all the above in service of the first part of my cluster of ideas: “The ethicist’s role as consultant in hospital as best understand as brokering the progress of technical death and bodily death in challenging cases so that they occur at the same time, to the degree that that is possible.”¹³

II

So now the question is, if you grant that the above is the case, why should we as students of religion and secularization care about this? First of all, for my immediate purpose, the argument is this that “from her standpoint, built out of dozens and dozens of consultations, the ethicist who is looking (that is to say, the ethicist schooled in social science), will be in a unique position to see and say useful things about secularization and religion in the hospital.” The only way to substantiate this claim is to go ahead and say useful things. But it is worth adding that if her position is unique, it is because of the wide variety of conversation partners consulting ethicists will have doing consultations all across the hospital. As will be evident in the anecdotes related below, I am speaking here of a wide variety of conversation partners, a wide variety of topics, and, if you will, a wide range of “urgencies”—from policy questions that may unroll only through months and months of excruciatingly endless committee meetings to terse conversations with surgeons in emergent situations.

Secondly, the development of the role of ethicist indicates a setting aside, or a more precise specialization of chaplaincy. Let me put it this way: Why isn’t the chaplain called in to negotiate the convergence of technical dying and bodily dying? My argument would be the simple-minded one that the chaplain, representing religious interests, is tied inextricably to the interests of bodily dying. And is, by definition, excluded from assessing the process of technical dying by virtue of being associated with religion.¹⁴ The ethicist on the other

hand, being neither fish nor fowl, can go back and forth between the technical and the bodily. The ethicist has no stake in either side, supposedly. The ethicist is a broker.¹⁵

However: The organization that makes technical dying possible does, in the last analysis, pay the ethicist. Provides, if you will, the broker’s commission. So, to that degree, the ethicist has a stake in the ongoing enterprise of producing technical death. But in a given case—unlike the medical teams treating the patient or those friends and family accompanying the patient—the ethicist (theoretically, ideally) has no special interest in the outcome going either way.¹⁶

This unusual positioning of Ethics is *not*, one wants to emphasize, due to the ethicist having some special expertise—or wisdom (!)—as regards right and wrong, good or bad; or some unique authority in the presentation of ethical judgments; or some distinctive capacity to make sound ethical decisions.¹⁷ In this way, the ethics consultant is *not* the direct heir of the priest or the minister or the imam or the spiritual master. If we consider the ethicist a negotiator, a commentator, a broker, we might expect (or hope . . .) to find a certain practical wisdom. But only such limited wisdom as is involved in getting people together, getting them to talk to one another—getting people to look at what is right in front of them.

III

Now then, what sort of useful observations might the ethicist have to make about secularization in the hospital, especially as regards the secular discourse, as it is practiced in the hospital? The primary one is that as an ethicist, one talks to many

¹³ It would not be too much of a stretch to say that roughly 95 % of ethics consultation cases in the hospital where I work involve negotiations back-and-forth between those with interests in the technical body and those with interests in the patient’s body. More specifically, 100 % of end-of-life decision-making, DNR/DNI, and “medical futility” cases count here. And roughly 80% of goals-of-hospital-care and surrogacy decision-maker cases as well.

¹⁴ In terms of being drafted for ICU negotiations, chaplaincy is hampered by the same constraints noted in the text for nursing and social work. And then there is the scorn on the part of many physicians for the “crying and dying” service. Here again I recommend Cadge (2012).

In this vein: After my first or second official consultation, my supervisor told me that offering and then giving a hug to an obviously distressed family member after a family conference was “the sort of thing a chaplain does, and not appropriate for an ethics consultant.”

¹⁵ DeVries and Conrad have argued that ethics consultation is essentially social work (1998). But the problem with this is two-fold. 1) Social workers have a vast array of technical knowledge ethics consultants can’t begin to match. They know stuff; we don’t. They have skills to which we should consistently defer. 2) As argued in the text, ethics consultants engage in negotiations that social workers are not called into. In our many hospitals, social workers are mostly delighted to turn those situations over to Ethics. And in others, social work sees Ethics as competition. Studies of these dynamics would be most instructive.

¹⁶ There is extensive discussion of this topic. But see especially Sharpe (2007). And then also “DeVries and Bosk (2004), DeVries and Keims (2008) and Elliott (2007).

Going further: “Bioethics is the public relations division of modern medicine, whether physicians (or bioethicists) like it or not.” (Imber 1998). And then, from a conversation with a private investigator who has much experience researching complex malpractice and workman’s compensation cases, of necessity a “lay ethnographer” of considerable talent: “You’re in risk management. That’s what you are—risk management.”

¹⁷ With this argument in mind, I remain terribly skeptical of efforts to delineate “core competencies” for consultant/negotiators like that found in Ausilio et al. (2000).

different tribes in the hospital: surgeons and intensivists; nurses and social workers; lawyers and discharge planners. And the impression one gets is not that of a single secular language, not that of a “default discourse”, that doesn’t reference the Divine as its foundational term, as Berger (2012) has it, but of a Babel of modernity’s languages.¹⁸ Here are four examples.

- 1) *Whose body is it?* A care conference is taking place at the bedside of a patient who has had major heart surgery for coronary artery disease with two vessels with more than 50 % blockage. Course complicated by cardiac arrest and significant stroke 10 days after the surgery. At the time of the conference, he is sedated and with continuous positive airway pressure (CPAP), and all agree he is not competent to make medical decisions even when unsedated. There are the interpretations of the status of the technical body:

Cardiac surgery says the CPAP settings are minimal, the expected rehabilitation period at the skilled nursing facility is less than 6 months, the stroke not catastrophic. “All is going as anticipated,” he says. “We should stay the course.”

Neurology says that while last Thursday things looked very grim, this Tuesday there is some improvement in reflexes and perhaps a bit of tracking. They conclude that the situation is more ambiguous than it has seemed 5 days before and recommend that we wait and see.

Critical Care says the CPAP settings are maximal, the rehabilitation period will be at least a year, that the patient will most likely never leave the nursing home, that the stroke is a major one. They want us to consider taking the patient off respiratory support, in keeping with wishes expressed in his Advanced Care Plan.

And then there are the understandings of the activity surrounding the patient’s body:

Social Work has in hand said Advanced Care Plan that the patient signed 2 days before the surgery. It says that he does not want to be kept alive by artificial respiratory support and does not want to be dependent on others for daily care. But it also names the patient’s sister as his healthcare decision-maker. The social worker feels the Advance Care Plan should be honored and respiratory support withdrawn.

The sister says she will take care of the patient, whether he wants to be taken care of or not. She says he wants to live, even if he does not know it.

Ethics is very puzzled, and is dealing with what may best be called a “jumbled” body—one that is in desperate need of negotiation if there is to be any coherent body at all. All these discourses are secular. Perhaps the medical teams are speaking the same language, but at best three strikingly different dialects of it. Social work is speaking legal language and patient advocacy. And the family has taken the imperious speech of family care and togetherness.

- 2) *The individual vs the aggregate.* The nurse versus the physician. N: “We are torturing this patient. The feeds make her miserable. She’s aspirating. She’s always agitated and is inconsolable.” P: “In cases like this, when we see numbers like the lab is giving us here, we know that 40 % will go on to have such-and-such capacity. And the literature suggests that the this condition is not irreversible. Clearly the prognosis is not good. But we can’t rule out recovery. I’m not ready to give up on this baby.” Anspach (1987, 1993) has done a brilliant job illuminating the differences in perception by those caregivers who work with individual instances, case-by-case (bedside nurses), and those who work by aggregates, with percentages, studies, and cohorts (attending physicians). Both secular languages concerned with the “technical body.” But ones with, if you will, different grammars.
- 3) *Law and Medicine.* A patient with gangrene in her lower leg has been ruled not to have capacity to make her own medical decisions. No family is available to serve as a surrogate decision-maker. The patient is refusing surgery, even though the surgery team feels sepsis will likely kill her in the next 24–48 h. Here we have medical language and legal language over-lapping. Ethics is called, a physician is appointed surrogate, the surgery is done, the patient survives and is very pleased to be going back to her favorite nursing home.
- 4) *Administration and Bureaucracy.* Ethics is constantly on the phone with the General Counsel’s office and occasionally with the Risk Management team. But almost never on phone with insurance or accounting, whose only interest is in the technical body, insofar as it is the “billable body”. We are not privy to discussions in accounting about insurance, fee structures and the like. (Though one does occasionally hear rumors about billing codes being negotiated: “Now Dr. X, couldn’t you consider re-coding that?”) But Ethics is regularly in meetings with social workers and discharge planner meetings where these discourses are regnant.

The four anecdotes I move to next will illustrate this phenomenon of secular Babel as it encounters religious pluralism. Or, if you will, aspects of secular dying versus religious dying. But I believe there is sufficient detail here to give initial

¹⁸ “. . . there now exists a powerful discourse which operates without recourse to religious definitions of reality. . . . I am not sure if this is the most felicitous phrase, but I would call it the *default discourse*: It is, as it were, the first call, made ‘naturally’ even by religious individuals before they deliberately switch to their particular ‘supernatural’ discourse.” Berger (2012: 315, his emphasis)

plausibility to my third cluster of points concerning secularized language in the hospital.

- a) There is a pluralism of secular discourses in our hospitals, such that, for instance, people speaking “respiratory therapy” cannot be guaranteed to understand people speaking accounting—and vice versa.
- b) Even so, we can identify “technical body talk” as the most privileged secular language of hospital.

IV

I want now to talk through four ethics consultations where bodily dying was engaged primarily as a religious matter. A typical chart note, preceding such a consultation request would be: “Family opposed to withdrawal of care for religious reasons.”

This will be a ground-eye view—the view of the person taking cases as they come. This is not about an ideal or theoretical picture of religious pluralism and ethics. It is not about policy, or even training. It’s about what one consultant did when faced with the human need for transcendence when confronting dying and death. Little or no time for study; no chance to call in the experts. (Whoever they might be.) Just do it as best you can and hope for a human connection, if nothing else. Pluralism and secularity on the ground, if you will.¹⁹

I would add, though, that born of necessity as it was, this approach has become for me a deliberately chosen, pragmatic stance. Even an existential one. The fewer theories, the less critique I carry into ethics consultations, the better they seem to go. So my plan has become: Stay still; pay attention; be there.²⁰

As a reminder—what I hope to do through these four anecdotes is support my contention that:

- a. Religious practice and belief enters the hospital with and through the physical body;
- b. There exists a phenomenon I will call “negotiated transcendence”, which is often the only shareable route to the sacred within the confines of the hospital. And that the ethicist may be called on to negotiate this as well.

In each of the four anecdotes below the technical dying process is being managed alongside bodily dying as understood

by a family with strong ties to a religious community or tradition.²¹ The divergence from the technical dying that threatened the convergence of interests and factors towards “a natural death” was in each of these cases perceived by the medical teams and by the families as explicitly and specifically religious. In each case the consultant brokered or was part of brokering a solution that brought the technical body in alignment with the religious body, such that a death occurred that was acceptable to the medical teams, the families, and (where indicated) religious authorities as well. More specifically, the technical dying and the bodily dying of the patients in question were coordinated with one another and, at the same time, a place was made for actions that preserved, confirmed or established the “religious body”, the body itself as sacred.²²

When speaking of religious families and religious communities having an interest, a stake, in bodily dying and a party to negotiations towards a “natural death”, as we are here, we need to keep reminding ourselves that religion is fluid, and complex beyond imagining. That practices vary from church-to-church, denomination-to-denomination, sect-to-sect, mosque-to-mosque, synagogue-to-synagogue—much less tradition-to-tradition, region-to-region, country-to-country. It is not as though an attending physician saying a family has “religious reasons” gets us very far at all, as negotiators.

Most of the back-and-forth in these negotiations is between frustrated teams and adamant families. So one says to the team: “Back off a bit. They need more time.” “Here is what the family is actually asking for.” Saying to the family: “What do you see when you look at her today?” “This is what the team is saying.” “What do you need that is not being done?”

In the languages and practices of the hospital there is so little footing for religious language and practice that we end up groping around for shards of shared speech for acknowledging transcendence. In an essentially secular setting with a plurality of religions, how can we get enough “purchase” to complete a sacred act—to act in any way to honor a body as sacred? I am saying that this space has to be negotiated, it has to be claimed, it has to be enacted.²³

It is too easy to assume, when thinking of religious pluralism, that what must be negotiated are beliefs or convictions: “What counts a miracle?” “How do you read this passage from Genesis?” But I would propose that just as often, and more fundamentally, we are negotiating proper action: “How shall this body be washed?” Or a fitting place: “Will my

¹⁹ For an account that captures the inherent messiness of the consultation enterprise see Andre (1998).

²⁰ Have we thought enough about the ethical responsibilities of ethics consultants to develop healing skills?

²¹ In keeping with the conventions of the modest genre of “consultation anecdote”, I shall use a combination of chart-note language and cadence, and commentary sentences. This being a rhetorical mode that has certain assets here. Cf. Bosk (2003: 229–235) on the different kinds of stories told by “insiders” and “outsiders”. The anecdote is an “insider” genre. See Chambers (1999) and Montgomery (2006) for very thorough treatments of the “case study” as a literary strategy and thinking method.

²² “The medical profession was the mediating institution between these sacred traditions of respect for the body, on the one hand, and the palpable, often painful realities of bodily living, on the other.” Imber (1998: 18).

²³ We have to keep asking ourselves: “What are we doing here? How does this work?” Whatever it is, it is not theology, not witnessing, not interpreting the law. See Engelhardt’s penetrating assessment of what he calls “the ordination of bioethicists (2002). More pointedly: “Are we simply manipulating sacred tongues in service of hospital agendas?”

baby's body be sent to the morgue?" So much power resides in these acts, and in the presence of the body seen and body touched, the body heard and smelled. We have to keep in mind not only the evidence constantly in front of us in the hospital, but the ritual practices of all of human history. The patient's body is the site where we look and wait, the site where miracle would be confirmed. The body born, as gift and miracle; the body restored, as gift and miracle.

The Sovereignty of God: Taking Back His Own

The Divergence of the Technical and the Bodily The patient's parents request that the child be nursed by her mother and be discharged home to die in her mother's lap. The medical team is uncomfortable with both of these requests.

Negotiation One Carefully measured feedings and tracking of weight gained and lost over against the mother and father's sense of the child's misery and of how much feeding is needed.

Negotiation Two Dying in hospital or in hospice care (medicalized death) as opposed to dying at home in mother's lap (understood in the light of the *Hadith*—God gives; and God takes back).

Negotiating Transcendence "You spoke beautifully about your child." "It is right you want your baby to die at home."

8/26. Consulted by team during morning rounds about African parents wanting to take their baby home "against medical advice" (AMA). Mother speaks no English and the father's English is rudimentary. Father and mother of patient concerned about baby's nasogastric tube (NG) feedings, which they feel are causing baby unnecessary suffering. Team on the other hand concerned about amount feeding and nutrition needed for appropriate weight gain at this age—and this child is not gaining but losing weight. They are likewise concerned about the child going home without hospice support to monitor the tube feeds. The team asked for Ethics to come talk with parents about these issues.

I had met the father 3 weeks earlier at a family care conference. While he was translating for his wife over the phone, the highly unusual and terribly awful syndrome the child had was explained. At the end of the conference the father said, "Thank you for taking care of my baby. I appreciate what medicine can do, and what all of you are doing for her. But God has given us this baby—and when he is ready, God will take this baby back, no matter what you do."

The attending physician thanked him for talking with us about his child. To close the meeting I said, "I would also like to thank you for speaking so beautifully about your baby." He passed me on the way out and gave me this most beautiful smile.

This was the man who wanted to take his baby home.

The full sense of the miracle, spoken and manifest, was the gift of the child. The power of God lived in the gift—and in God's corresponding ability and prerogative to take the child back, regardless of what secular medicine might try to do. The technical body was understood by the parents to be subordinate to the body given by God.²⁴

The parent's concern—quite legitimate as it turned out (confirmed by various members of the nursing staff and the Fellow on the case)—was that the forced tube feedings were causing the child to be miserable. When the father asked that the feedings be stopped or the amount in a given feeding

²⁴ *Sahih Muslim*. Chapter 20: THE MERITS OF ABU TALHA ANSARI

Anas reported that the son of Abu Talha who was born of Umm Sulaim died. She (Umm Sulaim) said to the members of her family: Do not narrate to Abu Talha about his son until I narrate it to him. Abu Talha came (home) ; she presented to him the supper. He took it and drank water. She then embellished herself which she did not do before. He (Abu Talha) had a sexual intercourse with her and when she saw that he was satisfied after sexual intercourse with her, she said: *Abu Talha, if some people borrow something from another family and then (the members of the family) ask for its return, would they resist its return? He said: No. She said: I inform you about the death of your son.* He was annoyed, and said: You did not inform me until I had a sexual intercourse with you and you later on gave me information about my son. He went to Allah's Messenger (may peace be upon him) and informed him what had happened. Thereupon Allah's Messenger (may peace be upon him) said: May Allah bless both of you in the night spent by you! He (the narrator) said: She became pregnant. Allah's Messenger (may peace be upon him) was in the course of a journey and she was along with him and when Allah's Messenger (may peace be upon him) came back to Medina from the journey he did not enter (his house) (during the night). When the people came near Medina, she felt the pangs of delivery. He (Abu Talha) remained with her and Allah's Messenger (may peace be upon him) proceeded on. Abu Talha said: O Lord, you know that I love to go along with Allah's Messenger when he goes out and enter along with him when he enters and I have been detained as Thou seest. Umm Sulaim said: Abu Talha, I do not feel (so much pain) as I was feeling formerly, so better proceed on. So we proceeded on and she felt the pangs of delivery as they reached (Medina) and a child was born and my mother said to me: Anas, none should suckle him until you go to Allah's Messenger (may peace be upon him) tomorrow morning. And when it was morning I carried him (the child) and went along with him to Allah's Messenger (may peace be upon him). He said: I saw that he had in his hand the instrument for the cauterisation of the camels. When he saw me, he said: This is, perhaps, what Umm Sulaim has given birth to. I said: Yes. He laid down that instrument on the ground. I brought that child to him and placed it in his lap and Allah's Messenger (may peace be upon him) asked Ajwa dates of Medina to be brought and softened them in his month. When these had become palatable he placed them in the mouth of that child. The child began to taste them. Then Allah's Messenger (may peace be upon him) said: See what love the Ansar have for dates. He then wiped his face and named him 'Abdullah. (my emphasis)

Sahih Muslim, by Imam Muslim, translation by Abdul Hamid Siddiqui, Book 31: The Book Pertaining to the Merits of the Companions (Allah Be Pleased With Them) of the Holy Prophet (May Peace Be Upon Him) (Kitab Al-Fada'il Al-Sahabah), Number: 6013.

Accessible at: <http://www.usc.edu/org/cmje/religious-texts/hadith/muslim/031-smt.php>.

reduced, the nurse on duty told him that the doctor had ordered what they were giving, and that she could not change the order. Only the doctor could do that and, no, he was not available. Technical body trumps patient's body.

I spent 20–25 min talking with father. Urged him not to leave the hospital until we could clarify the question of nutrition and tube feeds, and until we could make sure that hospice would be able to get the equipment necessary for medically appropriate home care. Acknowledged desirability of having their child die at home and endorsed their preference for this. Also discussed potential financial consequences of taking patient out of hospital AMA. Promised to advocate to get feeding plan changed and to try to speed up process of setting up home hospice. Father accepted these points and agreed that it is best for child to remain on the unit. Went with father into waiting room while he explained to mother. She gave her assent to this plan. At that point he said, "All we want is for our child to come home so she can die in her mother's lap."

8/27. Arrived in the NICU for morning rounds. The attending confided in me that he was concerned that the child was losing weight and that he did not want to "starve the child to death". Patient's father and I met with attending and patient's bedside nurse. We reviewed our conversation of the day before. Father re-stated his agreement with plan to keep baby on unit. He said he was "confused" the day before, but today sees reasons for keeping baby here.

Attending explained the team's concern about feeding and nutrition. In reply, the father said, "Wait. Let me show you. Big cup, your feedings. Small cup, baby's stomach. You pour big cup into small cup and it overflows. But mother's feeding is small cup into baby's small cup. No overflowing. Just many small cups." Ah!

Attending agreed to order 5–10 min feeds by mouth before proceeding to NG feeds. Nurse also given discretion about how much NG feed to push, this order to be placed immediately. Father very happy with this. Attending also assured father that team is working hard to get hospice in line for discharge home ASAP. Father pleased with meeting.

8/28. Received page from NICU Case Manager asking me to come meet with patient's father regarding today's discharge of patient home with hospice support. Met with father for

25–30 min. Discussed plan for discharge. Talked over nature of hospice services. Fellow and then attending came in and reviewed pertinent aspects of home medical care for child. Nurse provided father training in tube feeding. (Though neither fellow nor nurse nor I were under any illusion that the parents would actually do this once home.) The parents took the child home that day. The team was relieved, as was the family. The father expressed great appreciation for the care their child had received.

Approximately 2 weeks after their services were engaged, the home hospice nurse called to say that the child had died at home, apparently very peacefully.

She had been taken directly to the mosque to be properly prepared by the imam for burial.

The Sovereignty of God: Awaiting a Miracle

The Divergence of the Technical and the Bodily The placement of a "Do-Not-Resuscitate/Do-Not-Intubate" (DNR/DNI) order as a religious issue. Two members of the family see accepting the medical team's decision to place the DNR/DNI order as a sign that they are giving up on the possibility of a miracle, and in so doing giving up on God. For the team, placing the order is justified by the clear progression towards death of both the technical body and the perceptible body.

Negotiation One The body as the site of a possible healing miracle as opposed to both the technical and perceptible body understood by the team to be dying.

Negotiation Two The medical team as decision-maker about the body's care versus the family as decision-maker.

Negotiating Transcendence Bible discussion about suffering and miracles. Conversation about brutality of fate and not understanding what God was doing. About the limits of human power and release from the fury of suffering.

12/30. Page from a Nurse Practitioner (NP) in the Medical ICU (MICU). Three issues identified by the medical team: 1) code status; 2) communication with family about goals-of-care and possible shift to palliative track; and 3) clarify surrogate status. Team believes strongly that patient needs to be DNR/DNI given multiple co-morbidities, declining condition, and imminent onset of active dying process. Family resisting this move and reported by the team to be consistently contentious. Team unsure whether the father or the daughter is actually making the decisions about the patient's care. Thus on grounds of technical dying and clinical signs of bodily dying, the team "is ready". But the family "is not ready." The husband is not ready to let go of his wife. Their children are waiting for a miracle. "God can do miracles. Even when medicine can't save and says all is over. God rules over medicine."

Affect of the patient's husband experienced by members of the medical team as "threatening", "hostile", "aggressive". Husband is a very large man, who has had difficult and angry life. Not necessarily angry at staff but angry at the world and hurt by his life. (But then easy for Ethics to say, as I'm not there all day long dealing with him, like the nurses and house staff and NP's are.) Loss of his wife cutting very deep. "My wife went back to school 20 years ago at age 36 to get her bachelor's so she could teach. After seeing all kinds of people struggle in Job Corps." How much he loved her; how remarkable she was. He could see she was suffering and had little

strength left, and knew she would not want to live in anything near this condition.

Team is saying technical body is dying. Dad is stuck between his wife who he sees is suffering (patient's body, clinical signs) and his children who are counting on a miracle (patient's body, religious body), with daughter in lead and son a second. She especially vociferous about this. Daughter goes as far as to tell Ethics that she is worried that witnessing her mother's illness and suffering is weakening her father's faith, because he didn't seem to believe they should wait on (or count on) God to perform a miracle.

1/2: Talked with father about tough decisions and need to give his daughter guidance. Had discussion with daughter and son about times in Bible when God called on people to suffer and witness suffering. Not always miracles. Which were they being called to do now? Discussion moving back-and-forth between technical dying and bodily dying, with bodily death approached primarily through religious practices: praying for healing; and praying for the strength to keep hope alive and faith strong.

1/3: Visited for 15–20 with patient's son. Re-iterated that options for adjusting components of patient's care are getting more and more limited. And that her condition is terminal. Once more placed comfort care on the table. Again, back-and-forth between technical dying and bodily dying, but here observation of clinical signs has preeminence.

1/4: Checked in with MICU team. Vasopressor support for patient in place. Renal condition worsening. Spent 35–40 min with patient's daughter. Urged her to recall that beneath all her concerns about her mother's treatments and conditions lies the cancer that, fundamentally, will end her life. Urged her to consider that re-intubating her mother could be just as cruel as chest compressions and full heart resuscitation efforts, given mother's current condition. Urged her to talk with her father about the patient's level of suffering and quality of life. Her grasp of futility and of patient's suffering increases in each conversation, as does her brother's. Put hospice option on the table. More back-and-forth between technical dying and bodily dying, approached now through clinical signs and in the light family dynamics and perceptions.

1/6: Late afternoon: 60 min conversation with husband of patient in patient's room. Emphasized futility of further treatment for patient, her continuing suffering, and the need for DNI order. Urged explicit choice of palliative care. Talked about his children and his job and his marriage. About brutality of fate and not understanding what God was doing. At end, emphasized need for him to actively engage his daughter on these questions. Still more back-and-forth between technical dying and bodily dying, approached through clinical signs and family perceptions. But with the religious frame now explicitly re-engaged.

1/6, Evening: DNI placed by medical team. In light of both technical indicators and clinical signs.

Daughter switched over to chief mourner when code status changed. Continuing her role as chief spokesperson for family's religious interests, now as orchestrator of the death-bed vigil. Last time I saw the family, she was poised at patient's bedside with many family members present, including patient's mother, one sister and two grandchildren. Son functioning as greeter at the door—torn apparently between his sister and his mother. Dad lying in the dark on visitor's bed in back of room, his feet towards the patient's bed, blanket over his head.

1/9: Patient dies.

The Body as Sacred: Caring for the Gift

The Divergence of the Technical and the Bodily Team believes extubation is called for while the parents want to take the baby home on a ventilator. Parents want body only washed at their mosque by the imam, but team feels need to do some minor preparation of the body before it leaves the hospital.

Negotiation One Breath as a machine-driven and bodily phenomenon versus breath as sacred and given by God.

Negotiation Two Reconciling the “hospital body” and the sacred body.

Negotiating Transcendence 1) Beauty of the belief about the Divine Breath acknowledged, even though acknowledged it was not shared belief. Agreement that life is miracle. 2) Washing body under supervision of family. Not sending the body to the morgue, but keeping it in the room until the imam arrived.

Background Team has had ongoing conversations with parents about end-of-life and goals of care. Baby with multiple system failures. Parents have allowed the team to place a DNR/DNI order. But don't want him off vent, “for religious reasons” the attending's chart note says.

4/27. Met with parents at 10:00 a.m., after conversations over last 2 days with attending and medical team. Consult request came because team uncertain how to proceed with care because of the parents' religious beliefs. The parents have told the team that they are Muslim and that they have been talking with their imam. When the child dies, they plan to take him to the mosque for the preparation of the body for burial.

Can't get interpreter because telephone translator speaks a dialect of primary language that is less comprehensible than English to the father of the child. So we proceed—father, mother and aunt. Began to review situation, and talk with parents about their religious beliefs and child's limited care options. Parents do not want the child removed from the vent, and make it clear they want to take child home on the vent. I point out child receiving other forms of support—not just

respiratory. At this point in the conversation, I speak of life and breathe as sacred—and that this is miraculous and can be understood as gift of God. Say that though I do not share their faith, I understand the beauty and the power of this idea, and that I honor it.²⁵

What I am told later by their imam is that they were opposed to withdrawal because the heart was still beating, not because of any special sacred aspect of the breath. But what was going on in that moment was that we were reaching together for some shared language to speak about transcendence. This is what I mean by negotiated transcendence.

10:20: Nurse knocks on door of conference room, sticks her head in and says, “We need the parents in the child’s room. He is in crisis.” When we get to the baby’s room, bagging going on, but obviously futile. Aunt sits down and holds baby as he is dying. At 10:40 Fellow verifies death with stethoscope on the patient’s chest.

Family does not want the child’s body washed at all.²⁶ But we needed to remove peripherally inserted central catheter (PICC line)—and get basic hospital grunge off baby, who is sticky from tape and yellow with disinfectant, before releasing the patient’s body. Bedside nurse and consultant wash the baby, very briefly and very lightly, as family watched. Removed PICC line from heel, small amount of blood oozing out. Told them to let us know if they see anything being done they have problems with. Aunt covers child’s face. “We believe it is best not to look at the dead. It keeps us connected to them.” Baby swaddled with face covered.

Family did not want baby’s body going to the morgue. Very concerned that this not happen.²⁷ Ethics: “OK I will stay and hold baby here in this room until you return.” Imam arrives. Nurse takes baby downstairs and out front to his waiting car.

²⁵ This passage from the Qur’an 15:28–29 (in from Pickthall’s translation) was pointed out to me later:

And (remember) when thy Lord said unto the angels: Lo! I am creating a mortal out of potter’s clay of black mud altered,

So, when I have made him and have breathed into him of My spirit, do ye fall down, prostrating yourselves unto him.

Accessible at: <http://www.usc.edu/org/cmje/religious-texts/quran/verses/015-qmt.php>.

²⁶ I understand now that it is the custom for a same-sex member of the family to wash the body of the newly dead, but that this does not apply to younger children and infants. A ready-to-hand if rudimentary resource, which I only came upon well after the case above was resolved: Tanenbaum/Center for Interreligious Understanding (2009: 78–95, especially 94). It also turns out, to my considerable surprise, that similar though more abbreviated information is found in the Policies and Procedures manuals of a number of medical centers under the heading of “Pastoral Care.”

²⁷ I had likewise assumed teachings concerning the desecration of the dead, and that these teachings would address autopsy and organ removal, too. (Tanenbaum 2009: 94).

The Body as Sacred: Baptism and Release

The Divergence of the Technical and the Bodily Technical body and patient’s body read clinically are in sync, so the team sees the child as dying and ready for extubation. The parents, on the other hand, are able to speak directly of not being ready to let their child go.

Negotiation One Clinical signs of the patient’s body being ready for death that the teams are seeing side-by-side with the parents’ ignorance of those signs and their desire to hold on to their child for as long as possible.

Negotiation Two When parents request that their baby be baptized, finding out how to make this happen in the NICU.

Negotiating Transcendence Performing the baptism.

Background There had been a long series of conversations among teams and with parents about the desirability of doing a tracheotomy, and about the kinds of breathing support available and advisable. Parents had also been, over prior 10–12 days, been very reluctant to agree to DNR/DNI order. Parents were struggling to grasp the technical body of their child as presented to them by the medical team. And also, in turn, trying to grasp the bodily dying of their baby. And trying to translate how that would relate to the technical death of the technical (intubated) body.

The consultant received a call from the Neonatal ICU (NICU) critical care team reporting that the patient had self-extubated. Patient had been re-intubated in crisis and resuscitated. Parents were present when the consultant arrived. They were in obvious and appropriate distress over these developments, and yet unwilling to consider deliberate extubation, even though they could see for themselves that the child was declining rapidly. As commonly happens, their eyes were going back and forth between the monitor and their baby, watching the oxygen saturation numbers drop, despite maximum respiratory support.

As the afternoon goes on, they finally agree to extubation, should the team think that appropriate. And then about 15 min later they change their minds. Mother: “No, we can’t let him go. Maybe there is a small chance.” Ethics: “I know this is the most terrible position parents can be in. You can see that he is ready; you have said that. I know you aren’t ready. But you can’t keep him here just for yourselves.” M: “I know. OK, I see. But we want him baptized. Once that is done, we will agree to have the tube taken out.” Here all faced the necessity of balancing the technical body and the technical death with the creation of the sacred body by the rite of baptism.

Unfortunately, however, no chaplain will be available in timely fashion, given the child’s rapid decline. Ethics: “In the church where I grew up, anyone was allowed to baptize,

especially in an emergency. What about your church?” Mother: “Yes. It’s the same in mine.”

E: “Ok, we are going to baptize this baby.” Asks for water in small container. Nurse (who has not been aware of the conversation): “What kind of water? How much do you need?” E: “Any kind. Just a small vial will do.” (The consultant being an Episcopalian by upbringing, he did not feel much water was required. Besides, immersing an intubated child would have been far beyond his ken.)²⁸ Together the consultant and the parents put their hands on the child, water was poured on his forehead and the three gave thanks for the baby’s life, offering a blessing on the baby’s death.

The child was then extubated and died within the hour, lying between his parents in a hospital bed with them curled around him. In our terms, the technical death, the bodily death and the religious death coinciding as well as they possibly could have. Not just a natural death, but a beautiful one.

V

We derive four major lessons from the above discussion of the encounter of the religious pluralism with secularization within the contemporary Western hospital.

Lesson One We are inclined to think of religious pluralism in terms of varieties of belief, in terms of claims about that which is true and that which is false: “I believe in a Higher Power.” “I believe miracles can happen.” “We are waiting on a miracle”. (“Are you a Christian?” Where this means “What do you believe?”, not “Do you love your neighbor?”) But this focus on belief, and what is true and false, is a decidedly intellectualized approach to religion, which yields a manifestly inaccurate view of pluralism. I would argue, instead, that religious pluralism manifests in hospital especially around actions focused on the body. *Religion enters the hospital in the body, not in doctrine.* Religious pluralism means there are many religious ways to understand body, as can be seen quite clearly in divergent funerary practices, as the dead body itself is passed from the hospital into the care of families and religious communities.

Lesson Two Despite the pluralism of secular discourses in hospital, we are justified in saying that the language of technical body is the shaping discourse. We claim this all the while acknowledging that there are many “dialects” of technical body talk, depending on which tests or machines or procedures are involved, and whatever data or imagery they

produce. Likewise, we recognize that technical body talk is distinct from the secular tongues of physiology, organic chemistry and the basic sciences. In ethics consultation negotiations we *always* have to have the technical body included, though many other languages—secular and religious—may come and go as the back-and-flow proceeds.

Lesson Three We cannot but be impressed, as Peter Berger has emphasized, by the durability of religion in face of modernity and secularization.

Here I just want to propose a significant addendum to a theory of pluralization: *A default secular discourse co-exists with a plurality of religious discourses, both in society and in consciousness.*²⁹

But the fact that there is a wide variety of secular discourses in the hospital carries an implication that is important for additional understanding of the encounter of religion and the secular, both in general and in the hospital in particular. The hospital ought to be of interest not so much as a temple of modernity, reflecting the monism of modernity’s self-image, but as Babel with its confusion of tongues and perpetually falling, without ever quite hitting the ground.

Modernity was miscast and misunderstood, by its own theoreticians and historians, as monolithic and inevitable. But the situation was and is far more complicated than secularization theory depicted. (Berger 2012) Indeed, it could be argued that a significant factor in the long-term and unexpected survival of religious life in the face of modernity and secularization is the existence of this plurality of secular discourses. That is, it turns out that *the secular is as plural as the religious*.

Consider two quick examples of this encounter of pluralisms. In our area Amish families have distinct impact on secular accounting discourse in the hospital, as well as on secular medical discourses. The communities in our area prefer to carry no debt and refuse health insurance. Additionally, they seek to avoid “high technology”, though the definition of this tends to fluctuate, especially, for instance, as a child gets sicker and sicker.³⁰ On the other hand, Jehovah’s Witnesses have here, as they do most places, very precise specifications for interactions with the secular legal system and the secular healthcare system. In both cases, distinct religious discourses and a plurality of secular discourses interact and influence one another in a myriad of ways secularization theory could not have predicted.

²⁹ Berger (2012: 316).

³⁰ There are those who feel that these two factors together result in inferior care for some Amish families. But such a conclusion would depend on sorting out with some precision what the goals are of the care of which we are speaking. In terms of certain technical goals, perhaps the care is not “as good.” But doesn’t the very existence of Amish communities constitute an intentional challenge to the assumption that the technical can guarantee the achievement of goals that are suitable final ones for human life?

²⁸ And, with an element of what might be called “residual piety” (following the lead of Philip Rieff), the consultant did not say “In the Name of . . .”, thinking that ritual formulation only for priests to speak, but rather “In the presence of . . .”). See Rieff (1972: all Index entries for “Piety” and “Pietist(s)”).

Lesson Four The hospital collects discourses and practices around death, and the bodies of the dead. “Bodily death” has long been the province of religious language and practice—forever as far as we can tell. “Technical death”, on the other hand, may be thought of as an aspect of “modernization”, as an effort to exclude religion from the province of dying.³¹ But the obvious presence of a plurality of religions in the hospital shows that this effort to have “technical death” reign over “bodily death” has failed here, as elsewhere. The hospital sits on the boundary of the sacred and profane necessarily.³² So let us close with a straightforward thesis: *Religion will remain in the hospital as long as the hospital is a place where bodily dying and death occur.*³³

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³¹ This being but one among many efforts to secularize death. (Actuarial tables and automotive safety tests being others, e.g.)

³² “The medical profession—insofar as it remains a mediating institution between sacred acknowledgment of the body and scientific uses of it—is constantly tested in how it makes connections between means and ends, creating the many forms of public ambivalence about medical practice in its wake.” Imber (1998: 25).

³³ Berger (1967: 51). “The power of religion depends, in the last resort, upon the credibility of the banners it puts in the hands of men as they stand before death, or more accurately, as they walk, inevitably, toward it.”

Also Berger (1967: 80). “To repeat, every human order is a community in the face of death. Theodicy represents the attempt to make a pact with death. Whatever the fate of any historical religion, or that of religion as such, we can be certain that the necessity of this attempt will persist as long as men die and have to make sense of the fact.”

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